

Wardle Editorial Canon

Standards for Governed Editorial Artifacts

Part III — Clinical Case Standards

1. Purpose

Clinical Case Standards define how cases are authored, generated, revised, evaluated, approved, made playable, published, and maintained within the Wardle Editorial Operating System.

A Wardle clinical case is not merely a collection of symptoms followed by a diagnosis.

It is a structured educational experience designed to develop diagnostic reasoning through progressive disclosure.

Every case should require the learner to:

- interpret clinical information
- maintain and refine a differential diagnosis
- recognise discriminating findings
- respond to changing diagnostic probability
- identify the most defensible diagnosis
- learn from the reasoning process after completion

The purpose of case governance is therefore not only to ensure that the final answer is correct.

It is to ensure that the path to that answer is clinically coherent, educationally intentional, appropriately difficult, and safe for learner exposure.

2. The Clinical Case as an Educational Artifact

A clinical case is an educational expression of a governed diagnosis.

It represents one possible presentation, reasoning pathway, learner level, clinical setting, and educational emphasis.

A case does not define the diagnosis.

It teaches selected aspects of the diagnosis.

Multiple cases may teach the same diagnosis while differing in:

- age group
- clinical setting
- disease stage
- presentation
- difficulty
- diagnostic mimics
- reasoning objective
- escalation requirement
- management context
- clue progression

Case diversity should increase educational coverage rather than produce superficial variation.

Changing demographic details alone does not create a meaningfully distinct case.

3. Case Purpose

Every case must possess an explicit educational purpose.

The purpose should identify:

- what reasoning skill the case teaches
- which learning goal it supports
- which differential challenge it presents
- which discriminator the learner should recognise
- which misconception it prevents
- which escalation or management decision it reinforces

A case should not exist solely because a diagnosis lacks inventory.

Case generation should respond to an identified educational need.

Examples of valid purposes include:

- teaching early recognition of bacterial meningitis before advanced neurological deterioration
 - distinguishing chronic pulmonary aspergillosis from pulmonary tuberculosis
 - recognising unstable angina despite normal cardiac biomarkers
 - identifying scaphoid fracture despite normal initial radiography
 - distinguishing Addison disease from nonspecific chronic fatigue syndromes
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4. Case Identity

4.1 Stable case identity

Every clinical case must possess a stable identity independent of its current revision.

The case identity allows Wardle to preserve:

- revision history
- review history
- validation history
- publication history
- learner attempts
- scheduling history
- differential mappings
- AI draft history
- reasoning assessments

Editing a case should normally create a new revision rather than replace its historical identity.

4.2 Case title

The case title should support editorial identification without revealing the diagnosis to learners prematurely.

Editorial titles may be descriptive, but learner-facing titles must not expose the answer.

A case title should not:

- state the diagnosis directly before play
- contain a pathognomonic phrase that makes reasoning unnecessary
- imply an unsupported syndrome
- exaggerate the clinical presentation
- substitute narrative marketing language for clinical clarity

Editorial systems may display the diagnosis separately from the learner-facing title.

4.3 Diagnosis relationship

Every case should resolve to a governed Diagnosis Registry entry.

The relationship may initially be:

- matched
- review required
- unresolved
- requiring a new registry entry

A case with unresolved diagnosis identity should not become publishable.

The proposed diagnosis text should remain preserved even after registry mapping so that the original authorial intent remains auditable.

5. Case Source and Provenance

A case may originate through:

- AI generation
- manual authoring
- administrative editing
- restoration
- migration
- adaptation from an earlier case

Source describes origin.

It does not establish quality or authority.

Every case should preserve:

- original producer
- generation or authoring context
- diagnosis used
- relevant editorial brief
- reasoning path used
- teaching rules applied
- revision history
- validation history
- reviewer decisions

AI-generated and manually authored cases should undergo the same clinical and educational quality standards.

6. Case Revision

6.1 Purpose

A Case Revision is a versioned snapshot of the case at a particular point in its editorial history.

A revision should preserve the complete case content required to understand what was reviewed or published.

This includes:

- title
 - date or scheduling context where relevant
 - difficulty
 - history
 - symptoms
 - laboratory data
 - clues
 - explanation
 - differentials
 - diagnosis mapping
 - source
 - publication track
-

6.2 Current revision

Every actively governed case should identify one current revision.

Editorial actions such as validation, review, approval, and publication should apply to the current revision or explicitly identify another revision.

Approval should never apply ambiguously to a mutable case record.

6.3 Revision creation

A new revision should be created when a material change affects:

- clinical facts
- clue wording
- clue order
- explanation
- diagnosis

- differential list
- reasoning objective
- difficulty
- educational purpose
- management guidance
- investigation findings

Minor non-substantive corrections may be handled according to implementation policy, but no correction should erase a previously reviewed or published state.

6.4 Revision comparison

Reviewers should be able to determine:

- what changed
- why it changed
- who or what produced the change
- which previous issue it addresses
- whether the change affects reasoning
- whether revalidation is required

A revision should not be approved solely because an earlier revision was approved.

Material revisions require renewed assessment.

7. Case Structure

A Wardle case may include:

- introductory history
- symptoms
- physical examination
- vital signs
- laboratory findings
- imaging
- pathology
- clinical progression
- treatment response
- contextual clues
- final explanation
- differentials

These elements should be organised according to diagnostic reasoning rather than arbitrary data categories.

8. Case Opening

8.1 Purpose

The opening establishes the clinical problem while preserving sufficient uncertainty for reasoning.

It should communicate:

- patient context
- presenting complaint
- relevant timeline
- initial severity
- clinical setting

8.2 Quality standard

A strong opening should be:

- clinically plausible
- concise
- sufficiently informative
- not prematurely diagnostic
- appropriate to the target difficulty
- free from irrelevant narrative detail

The opening should create a diagnostic problem rather than tell a story for its own sake.

8.3 Narrative restraint

Cases should avoid excessive narrative language that does not contribute to diagnosis.

Details should exist because they:

- alter diagnostic probability
- establish severity
- clarify chronology
- distinguish a mimic
- guide investigation
- support management
- strengthen realism

Narrative engagement should emerge from clinical uncertainty, not decorative prose.

9. Clinical Clues

9.1 Definition

A clue is a discrete unit of clinical information revealed during case progression.

A clue may represent:

- history
- symptom
- examination finding
- vital sign
- laboratory result
- imaging result
- pathology result
- risk factor
- treatment response
- clinical course

Each clue should change, refine, confirm, or challenge the learner's diagnostic reasoning.

9.2 Clue necessity

Every clue should justify its inclusion.

A clue should perform at least one function:

- broaden the differential
- narrow the differential
- support the target diagnosis
- weaken a mimic
- identify severity
- reveal a complication
- trigger escalation
- guide management
- test interpretation of uncertainty

Clues that do not meaningfully affect reasoning should be removed or consolidated.

9.3 Clinical accuracy

Every clue must be clinically compatible with:

- the target diagnosis
- the case stage
- the patient context
- the severity
- the stated timeline
- other clues

A clue may be atypical where educationally intentional, but the case should explain its significance.

9.4 Clue granularity

A clue should contain one coherent unit of clinical information.

It should not combine unrelated findings simply to accelerate the case.

Overloaded clues make it difficult to assess which information changed diagnostic probability.

9.5 Clue wording

Clue wording should be:

- clinically precise
- concise
- grammatically complete
- interpretable without hidden assumptions
- free from unnecessary explanation

The clue should present the finding.

The breakdown should explain its diagnostic significance.

A clue must not include its own reasoning explanation.

10. Clue Progression

10.1 Purpose

Clue progression describes how diagnostic understanding changes as information is revealed.

A strong case should create meaningful movement from uncertainty toward justified diagnostic confidence.

The learner should not know the answer from the first clue unless the case is intentionally basic.

The learner should not remain unable to distinguish the diagnosis after the final clue.

10.2 Diagnostic states

At each clue stage, the case may be evaluated according to:

- leading diagnosis
- active differentials
- eliminated mimics
- unresolved mimics
- confidence change
- emerging discriminators
- remaining ambiguity

Clue progression should be assessed across the complete sequence rather than clue by clue in isolation.

10.3 Progressive disclosure

Clues should generally progress from:

- broad clinical presentation
- contextual refinement
- examination or physiological evidence
- focused investigation
- discriminating or confirmatory evidence

This sequence is not mandatory.

Alternative sequences are acceptable where they reflect realistic clinical reasoning.

10.4 Diagnostic lock-in

Diagnostic lock-in occurs when one clue makes the answer effectively certain.

A lock-in clue may be appropriate near the end of the case.

Premature lock-in occurs when the diagnosis becomes obvious before meaningful reasoning has occurred.

The expected lock-in point should be compatible with case difficulty.

10.5 Premature leakage

Premature leakage occurs when a clue reveals the diagnosis too early through:

- pathognomonic wording
- named diagnostic criteria
- an overly specific test
- a disease-specific treatment
- explicit specialist terminology
- a diagnosis embedded in imaging or pathology wording

A clinically accurate clue may still be educationally inappropriate if introduced too early.

10.6 Unresolved ambiguity

A case has unresolved ambiguity when the available clues support more than one diagnosis without a defensible discriminator.

Ambiguity may be educationally intentional during the case.

It should not persist at the final decision point unless the case explicitly teaches uncertainty.

11. Clue-Discriminator Annotation

11.1 Purpose

A Clue-Discriminator Annotation records how a specific clue weakens or eliminates a competing diagnosis.

It creates an explicit link between case content and differential reasoning.

11.2 Required elements

An annotation should identify:

- case
 - clue order
 - eliminated or weakened diagnosis
 - discriminator
 - reasoning
 - elimination strength
 - educational value
 - reviewer state
-

11.3 Elimination strength

Elimination strength should describe how strongly the clue affects the differential.

Possible interpretations include:

- weak
- moderate
- strong
- decisive

A finding should not be described as decisive unless it genuinely rules out or overwhelmingly disfavors the competing diagnosis in the case context.

11.4 Educational value

Educational value describes whether the discriminator is worth teaching.

A discriminator may be clinically valid but educationally weak if it is:

- excessively obscure
 - dependent on unavailable resources
 - not generalisable
 - irrelevant to the intended learner
 - unlikely to improve future reasoning
-

12. Clue Revision Draft

12.1 Purpose

A Clue Revision Draft proposes a controlled change to a case clue.

It allows generated or editor-authored improvements to be reviewed before altering the governed case revision.

12.2 Draft contents

A clue revision draft may contain:

- original clue
 - revised clue
 - proposed added clue
 - rationale
 - expected effect
 - clue position
 - source audit
 - review status
 - reviewer decision
 - application history
-

12.3 Expected effect

The expected effect should state what the change is intended to improve.

Examples include:

- delay diagnostic lock-in
 - strengthen discrimination from pulmonary tuberculosis
 - correct a mismatch between fever and cardiac findings
 - add missing escalation evidence
 - remove unsupported laboratory data
 - improve clue coherence
 - reduce ambiguity
 - increase educational value
-

12.4 Review and application

Approval of a clue revision draft does not itself modify the case unless the system explicitly applies the revision.

The system should distinguish:

- draft accepted
- draft rejected
- changes requested
- draft applied
- draft superseded

Application should create or contribute to a new case revision where the change is material.

13. Clinical Reasoning Objective

Every case should possess an identifiable reasoning objective.

Examples include:

- differential discrimination
- early-presentation recognition
- escalation recognition
- complication recognition
- management contrast
- resource-limited reasoning

The reasoning objective should determine:

- clue selection
- clue order
- required differentials
- explanation emphasis
- expected learner action
- difficulty

A case that attempts to teach every aspect of a diagnosis may become unfocused.

14. Differential Diagnoses

14.1 Purpose

Case differentials define the clinically plausible alternatives that frame the diagnostic challenge.

They should not be generic lists appended after case completion.

They should reflect the actual uncertainty created by the case.

14.2 Differential quality

A case differential should be:

- clinically plausible
 - relevant to the presented findings
 - appropriately difficult
 - educationally useful
 - resolvable using the available clues
 - mapped to a governed diagnosis where possible
-

14.3 Differential roles

A differential may serve as:

- primary mimic
- general differential
- important exclusion
- teaching differential

These roles should not be treated as equivalent.

A primary mimic usually requires explicit discriminator coverage.

An important exclusion may require escalation or safety reasoning even if less likely.

14.4 Differential restraint

A case should not include every possible diagnosis.

Excessive differential lists reduce clarity and make review difficult.

The case should prioritise alternatives that:

- are genuinely plausible
 - represent common confusion
 - are dangerous to miss
 - teach an important discriminator
 - align with the case purpose
-

15. Differential Mapping

15.1 Purpose

Differential Mapping resolves case differential text to Diagnosis Registry identities.

This supports:

- graph integration
 - duplicate prevention
 - reasoning analysis
 - coverage assessment
 - consistent terminology
 - cross-case comparison
-

15.2 Resolution states

A differential mapping may be:

- resolved
- ambiguous
- unresolved
- rejected

These states describe mapping confidence.

They do not independently determine whether the differential is clinically appropriate.

15.3 Mapping review

A reviewer should confirm:

- correct registry diagnosis

- clinical equivalence
- absence of inappropriate merging
- correct source location
- sufficient confidence
- suitable differential role

Unresolved mapping should not necessarily block early drafting.

It may block final governance where the differential is central to the case.

16. Differential Link

A Differential Link represents a governed relationship between the case and a resolved diagnosis.

It should preserve:

- case
- case revision where relevant
- linked registry diagnosis
- differential role
- source mapping
- review state
- rationale where required

Differential links allow the editorial system to distinguish authored text from governed clinical relationships.

17. Differential Elimination

A case should demonstrate how major differentials are weakened or eliminated.

For each important mimic, reviewers should be able to identify:

- which clue affected it
- what discriminator was used
- how strong the elimination was
- whether confusion remains
- whether the explanation communicates the distinction

A differential should not appear in the final explanation without meaningful connection to the case.

18. Case Learning-Goal Coverage

18.1 Purpose

Case Learning-Goal Coverage evaluates how strongly a case supports a diagnosis-level learning goal.

The case should not be considered educationally sufficient merely because it is clinically correct.

It should contribute to defined educational intent.

18.2 Coverage elements

Coverage may include:

- learning goal identity
 - coverage strength
 - covered discriminators
 - missing discriminators
 - covered mimics
 - missing mimics
 - evidence source
-

18.3 Coverage strength

Coverage strength should reflect the degree to which the learner must engage with the goal.

A learning goal is not strongly covered merely because the relevant fact appears in the explanation.

Strong coverage usually requires the learner to use the concept during case reasoning.

18.4 Missing coverage

Missing coverage is not automatically a case defect.

A single case is not expected to teach every diagnosis-level goal.

It becomes a defect where the case claims to support a goal but omits its essential reasoning requirements.

19. Escalation Coverage

19.1 Purpose

Escalation Coverage assesses whether the case teaches recognition of clinical deterioration, urgency, or the need for higher-level intervention.

It is particularly important for diagnoses where delayed escalation could cause significant harm.

19.2 Escalation types

Escalation may include:

- emergency stabilisation
 - urgent referral
 - specialist review
 - intensive monitoring
 - definitive intervention
 - treatment failure
 - complication recognition
-

19.3 Coverage requirements

Where escalation is part of the case purpose, the case should include:

- evidence that escalation is needed
- appropriate timing
- a clear reasoning pathway
- clinically correct response
- explanation of why delay is unsafe

Escalation should not be added as an unrelated management note after diagnosis.

20. Case Explanation

20.1 Purpose

The case explanation converts the completed case into structured diagnostic education.

It should explain why the diagnosis is most defensible and how the clues altered the differential.

The explanation should not merely restate the case.

20.2 Core components

A high-quality case explanation may contain:

- diagnosis
 - concise summary
 - diagnostic reasoning
 - key findings
 - differential analysis
 - clue interpretation
 - investigation interpretation
 - management implications
 - educational conclusion
-

20.3 Diagnostic reasoning

The reasoning should show:

- how the presentation was initially framed
- which diagnoses were plausible
- which clues shifted probability
- which discriminator was decisive
- why the final diagnosis is preferred
- why major mimics are less likely

Reasoning should follow the actual clue progression.

It should not invent evidence that was not present in the case.

20.4 Key findings

Key findings should be selected because they materially support the diagnosis.

They should not be an indiscriminate list of every abnormality.

Each key finding should be associated with its clinical significance.

20.5 Differential analysis

Differential analysis should compare diagnoses using case-specific evidence.

It should avoid generic textbook contrasts unrelated to the presented patient.

A high-quality comparison answers:

- Why was the alternative plausible?
 - Which finding weakens it?
 - Which finding supports the target?
 - Is the alternative excluded or merely less likely?
-

20.6 Explanation safety

An explanation must not:

- contradict the clues
 - attribute one clue's meaning to another clue
 - invent findings
 - overstate diagnostic certainty
 - claim that a finding is pathognomonic when it is not
 - provide unsafe management guidance
 - obscure uncertainty
 - rely on descriptive diagnosis wording instead of the canonical diagnosis
-

21. Evidence Relationships

Cases may support diagnosis-level evidence relationships.

A case-supported evidence relationship should identify:

- diagnosis
- evidence node
- relationship type
- strength
- discriminator weight
- reasoning summary
- supporting case
- governance state

Case evidence should reflect what the case demonstrates.

It should not be treated as external clinical evidence or guideline authority.

22. Case Validation

22.1 Purpose

Case Validation evaluates whether a case revision satisfies defined technical, clinical, reasoning, and educational requirements.

Validation supports review.

It does not replace human approval.

22.2 Validation dimensions

Validation may assess:

- required content
 - diagnosis mapping
 - clue structure
 - playability
 - clue progression
 - reasoning coverage
 - evidence consistency
 - differential quality
 - explanation completeness
 - unsupported claims
 - publication requirements
-

22.3 Validation outcomes

Validation may produce:

- passed
- failed
- error

A validation error means the assessment could not be completed.

It must not be interpreted as a passing result.

22.4 Validation history

Each validation run should preserve:

- case
- revision
- outcome
- blockers
- warnings
- validator version
- execution time
- relevant quality signals

A newer case revision should normally require new validation.

22.5 Validation limitations

Validation can identify:

- missing structures
- policy violations
- reasoning inconsistencies
- high-risk patterns
- technical defects

Validation cannot independently establish:

- full clinical correctness
 - educational excellence
 - institutional approval
 - publication authority
-

23. Reasoning Draft Validation

Cases may also undergo specialised reasoning validation.

Reasoning validation may assess:

- trust score
- trust tier
- reasoning coverage
- evidence coverage
- discriminator coverage
- unsupported claims

- hallucination risk
- blockers
- warnings
- strengths
- recommendations

Trust tiers such as high trust, review required, low trust, or blocked are assessments.

They are not approval decisions.

24. Case Review

24.1 Purpose

Case Review is the governed human evaluation of a case revision.

It determines whether the case is clinically sound, educationally valuable, and suitable for its next lifecycle transition.

24.2 Review scope

Case review should assess:

- diagnosis identity
 - clinical plausibility
 - clue accuracy
 - clue progression
 - differential quality
 - discriminator quality
 - reasoning objective
 - explanation
 - learning-goal coverage
 - escalation coverage where relevant
 - AI contribution
 - validation findings
 - learner safety
 - publication suitability
-

24.3 Review decisions

A case review may result in:

- approved
- rejected
- needs edit

The decision should apply to an identifiable revision.

24.4 Review rationale

Every non-approval decision should provide actionable rationale.

Approval should also preserve a concise basis where the case contains atypical, complex, or high-risk reasoning.

24.5 Review completeness

A reviewer should not approve a case without access to:

- current revision
- complete clue sequence
- diagnosis
- differentials
- explanation
- validation results
- unresolved blockers
- material AI drafts
- relevant teaching intent

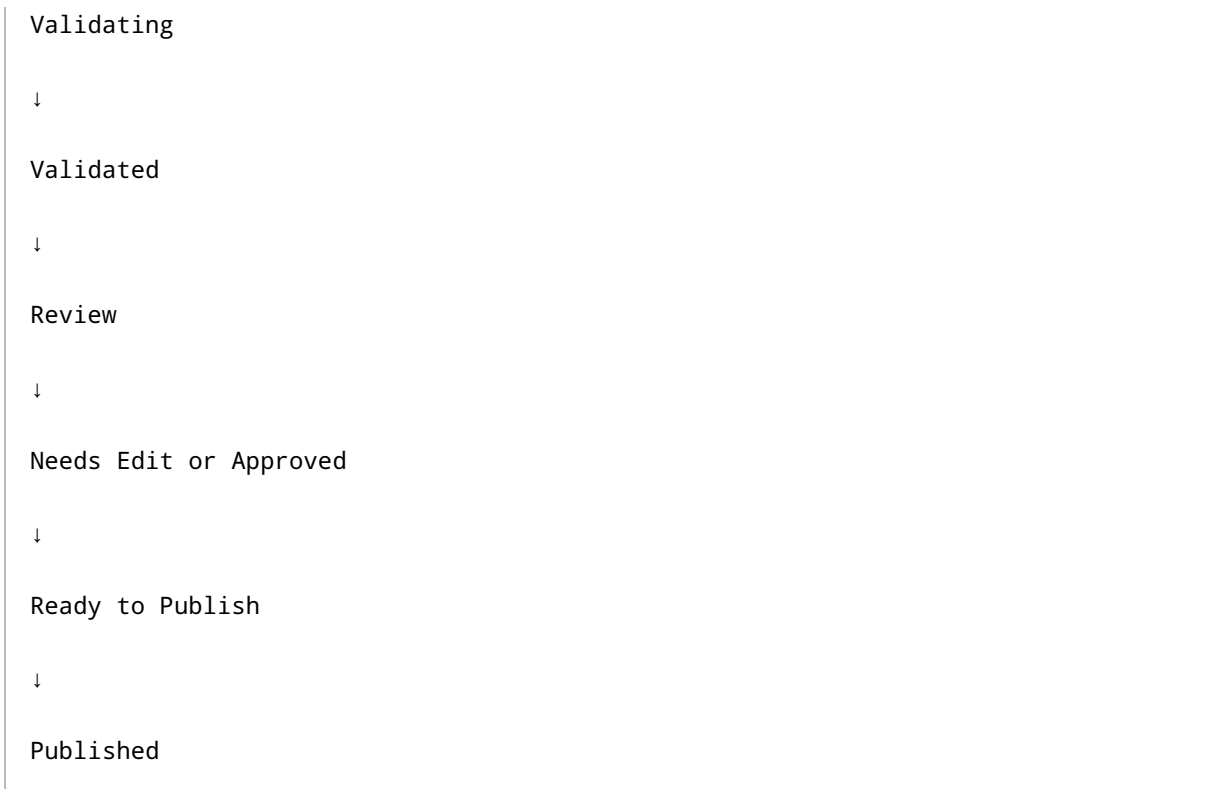
This complete context forms the Case Review Packet.

25. Case Editorial Lifecycle

The current conceptual lifecycle is:

Draft

↓



A case may also be rejected at the appropriate review stage.

25.1 Draft

The case is under authoring or generation.

It may be incomplete and should not be exposed to learners.

25.2 Validating

Automated or structured validation is in progress.

25.3 Validated

The current revision has completed validation successfully.

Validated does not mean approved.

25.4 Review

The case is awaiting or undergoing human editorial review.

25.5 Needs Edit

The case requires revision before approval.

25.6 Approved

The current revision has satisfied editorial review.

Approved does not mean published.

25.7 Ready to Publish

The case has satisfied the operational and diagnosis-level requirements required for publication.

25.8 Published

The approved case revision is authorised for learner exposure.

25.9 Rejected

The case has not been accepted into governed case inventory.

Rejection should preserve its history.

26. Playability

26.1 Definition

Playability means that the case can function correctly as a Wardle learning experience.

A playable case should possess:

- a governed diagnosis
 - sufficient clues
 - coherent clue order
 - valid answer matching
 - learner-safe content
 - a complete explanation
 - appropriate differential reasoning
 - no blocking technical defect
-

26.2 Playability and quality

A technically playable case may still be educationally poor.

A clinically sound case may still be technically unplayable.

Playability must therefore remain separate from:

- approval
 - educational quality
 - diagnosis activation
 - publication
-

26.3 Diagnosis dependency

A case cannot be operationally playable where its diagnosis is not permitted as a playable answer.

Both diagnosis-level and case-level playability must be satisfied.

27. Publication Eligibility

A case is publication-eligible when:

- its current revision is approved
- required validation has passed
- diagnosis mapping is resolved
- required differentials are governed
- blocking AI drafts are resolved
- clue revision drafts are resolved or explicitly deferred
- required explanation content is complete
- diagnosis-level publication dependencies are satisfied
- a permitted publication track is selected where required

Publication eligibility should be computed and explained.

28. Publication Tracks

A case may be intended for:

- Daily
- Premium

- Practice

Publication track affects distribution.

It should not alter the core clinical quality standard.

A Premium case should not be considered inherently more advanced.

A Daily case should not receive reduced governance because it is freely available.

29. AI Contribution to Cases

AI may contribute to:

- initial case drafts
- clue revisions
- differential suggestions
- explanation drafts
- reasoning analysis
- quality recommendations
- generation-context application

AI contribution should preserve:

- action type
- source issue
- input context
- generated output
- affected artifact
- review status
- reviewer decision
- review note

AI-generated changes must not modify the governed case silently.

30. Case Maintenance

Published cases should remain subject to review when:

- clinical guidance changes
- learner behaviour reveals ambiguity
- explanation errors are identified
- answer mapping changes

- diagnosis terminology changes
- new differentials become important
- clue progression is found to leak early
- management becomes outdated
- evidence relationships change
- quality standards evolve

Maintenance should create a new revision where the learner-facing case changes materially.

31. Case Retirement and Supersession

A case may be withdrawn, retired, or superseded where it is:

- clinically outdated
- educationally redundant
- persistently ambiguous
- based on deprecated terminology
- replaced by a stronger case
- inconsistent with current diagnosis governance
- unsafe for learner exposure

Retirement should not erase:

- learner history
 - publication history
 - review decisions
 - revision history
 - AI provenance
 - differential relationships
-

32. Quality Standard for a Governed Clinical Case

A high-quality Wardle case should be:

Clinically coherent

Every finding is compatible with the diagnosis, timeline, severity, and patient context.

Educationally intentional

The case serves an explicit learning and reasoning purpose.

Progressively discriminating

The clue sequence meaningfully changes diagnostic probability.

Appropriately uncertain

The case preserves useful ambiguity without becoming indeterminate.

Differentially grounded

Important mimics are plausible and distinguishable.

Explanation-aligned

The breakdown accurately interprets the clues actually presented.

Appropriately difficult

Difficulty arises from reasoning complexity rather than obscure trivia.

Learner-safe

The case avoids misinformation, unsupported certainty, and unsafe guidance.

Governed

The current revision has been validated, reviewed, and explicitly approved.

Maintainable

The case can evolve without erasing history.

33. Normative Statements

- Every clinical case **MUST** possess a stable case identity.
- Every material case change **MUST** be represented by an identifiable revision.
- Every case **MUST** resolve to a governed Diagnosis Registry entry before publication.
- Every case **MUST** possess an explicit educational or reasoning purpose.
- Every clue **MUST** contribute meaningfully to clinical interpretation, differential reasoning, severity, escalation, or management.
- Clues **MUST NOT** contain their own explanatory breakdown.
- Case explanations **MUST** interpret the clues actually presented.

- Case explanations **MUST NOT** invent findings or misattribute clue significance.
 - Important differentials **SHOULD** resolve to governed diagnosis identities.
 - Primary mimics **SHOULD** possess explicit discriminator coverage.
 - Clue progression **MUST** be evaluated across the complete sequence.
 - Premature diagnostic leakage **MUST** block approval where it defeats the intended reasoning task.
 - Unresolved final ambiguity **MUST** block approval unless uncertainty is the explicit educational objective.
 - Validation **MUST NOT** be treated as equivalent to human approval.
 - Approval **MUST** apply to a specific case revision.
 - Approved cases **MUST NOT** be treated as automatically publishable.
 - Playability **MUST** remain distinct from approval and publication.
 - AI-generated case content **MUST** undergo human editorial review.
 - Accepted clue drafts **MUST NOT** be treated as applied until the case revision changes.
 - Publication **MUST** identify the approved revision exposed to learners.
 - Published cases **MUST** remain maintainable and historically traceable.
 - Rejected, retired, and superseded cases **SHOULD** preserve their governance history.
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Closing Principle

A Wardle case is not successful merely because the learner can guess the diagnosis.

It is successful when every clue contributes to a clinically defensible progression from uncertainty to understanding, when important alternatives are meaningfully addressed, and when the final explanation teaches the reasoning that made the diagnosis credible.

Clinical Case Governance ensures that Wardle does not simply publish questions.

It publishes structured diagnostic reasoning experiences.